



Report ID: Hemo Check-20260628002518 | Patient: PT-20260628002514 | Physician: Dr. Sharma | Scan: Ultrasound DVT

COMBINED AI ASSESSMENT

High Blood Clot Risk Detected80.6% confidence

Scan model: 50.0% | Clinical model: 30.6% | Symptom score: 0.0%

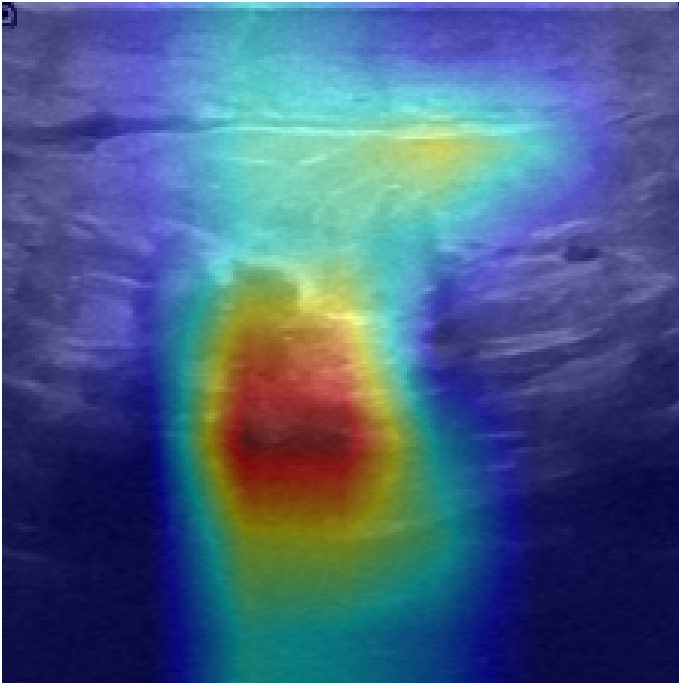
Overall risk classification:	HIGH
Overall confidence:	80.6%
Scan model contribution:	50.0%
Clinical model contribution:	30.6%
Symptom score contribution:	0.0%
Scan finding:	DVT Clot Detected
Scan confidence:	100.0%
Clinical risk level:	HIGH
Clinical confidence:	87.5%
Symptom severity:	NONE (0/100)
Urgency:	URGENT - within 24 hours
Recommended follow-up:	24-48 hours with specialist

ULTRASOUND DVT - IMAGING ANALYSIS

DVT Clot Detected100.0% confidence

Scan type:	Ultrasound DVT	Confidence:	100.0%
Model:	EfficientNet-B2	Inference:	0.51s
Risk level:	HIGH	Grad-CAM:	Generated

Grad-CAM Attention Heatmap



CLINICAL RISK ASSESSMENT

Clinical Risk: HIGH

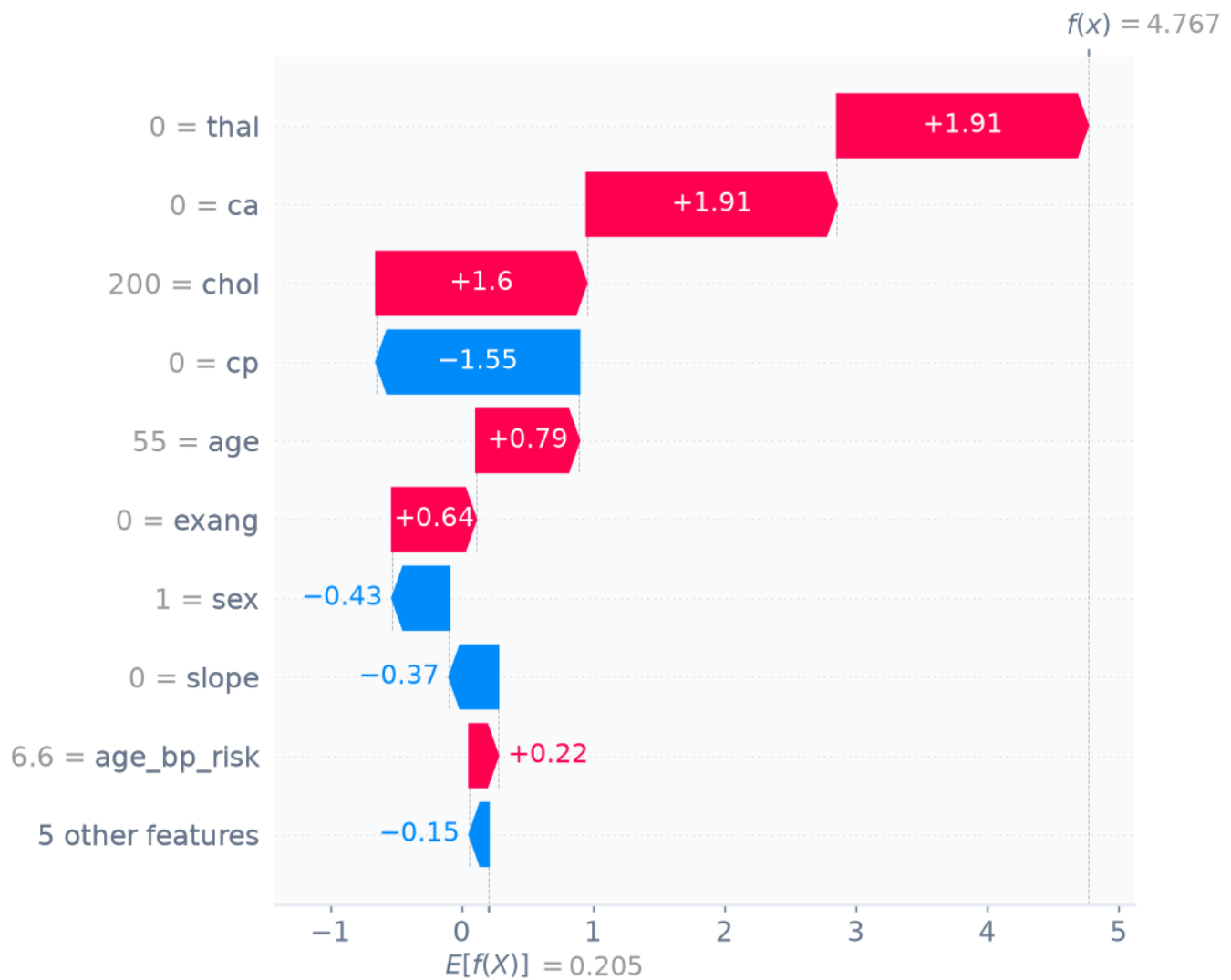
87.5% confidence

Age:	55 years	Blood pressure:	120 mm Hg
Sex:	Male	Cholesterol:	200 mg/dl
BMI:	25.0	Max heart rate:	150 bpm
Smoking:	Never	ST depression:	1.0
Diabetes:	No	XGBoost prob.:	99.19999694824219%

Symptom severity score: **NONE (0/100)**

AI EXPLAINABILITY - SHAP FEATURE ANALYSIS

Red bars increase clot risk; blue bars reduce risk. Bar length = magnitude of impact on the prediction.



CLINICAL RECOMMENDATIONS

1. Urgent clinical evaluation for deep vein thrombosis markers.
2. Immediate physician referral - do not delay.
3. Consider anticoagulation therapy consultation.
4. Repeat imaging within 24 hours if clinically indicated.
5. Review all current medications with treating physician.
6. Patient should not travel or remain immobile.

Urgency: **URGENT - within 24 hours**

Follow-up: **24-48 hours with specialist**

DISCLAIMER

This report is generated by an AI system for educational and research purposes only. It does not constitute medical advice and must not be used for clinical diagnosis or treatment decisions. Always consult a qualified healthcare professional for medical evaluation.